

HOW TO OPEN An Adult Care Home or Family Care Home in North Carolina

A step-by-step startup guide — licensing, staffing, and how you get paid

Regulatory basis: G.S. 131D · 10A NCAC 13F (adult care homes, 7+ beds) and 13G (family care homes, 2–6 beds)
Oversight: NC Division of Health Service Regulation (DHSR), Adult Care Licensure Section, with county Departments of Social Services

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The Big Picture

An adult care home in North Carolina is a residence that provides room, board, supervision, and personal care to adults — usually older adults or adults with disabilities — whose main need is a safe home with help through the day rather than the skilled medical care of a nursing home. North Carolina licenses these homes in two sizes: a family care home serves two to six residents and is regulated under 10A NCAC 13G, and an adult care home serves seven or more residents and is regulated under 10A NCAC 13F. The rules are written to be closely comparable, so the same fundamentals apply to both; the family care home is the classic path for an individual opening a small home.

Two agencies share oversight. The Division of Health Service Regulation (DHSR), through its Adult Care Licensure Section, issues the license, writes and enforces the rules, and conducts annual surveys. Your county Department of Social Services (DSS) is your local partner — for a family care home it is your first stop, and its adult home specialist walks you through the process, refers you to zoning, and collects your application materials. County DSS also administers the Special Assistance program and completes resident assessments, and it monitors your home at least quarterly after you open.

This guide walks the whole path in plain language, from forming your business to opening your doors, and it is honest about the part everyone asks about: how you actually get paid. It is general educational information, not legal advice — confirm the current rules with DHSR and your county DSS before you rely on any specific step, figure, or contact.

Two Homes, One Rulebook: Which Model Fits You?

North Carolina regulates both sizes comparably, but the model you choose changes your cost, your process, and your day-to-day. Use this to decide before you go further.

	Family Care Home (2–6)	Adult Care Home (7+)
Rule	10A NCAC 13G	10A NCAC 13F
Front door	County DSS adult home specialist	DHSR Adult Care Licensure Section (with county DSS)
Bed need / CON	Generally follows county DSS process	Governed by the State Medical Facilities Plan; CON may apply
Scale	House-scale; lower startup cost; classic owner-operator path	Larger operation; more staffing and physical-plant requirements
Administrator	Family care home administrator approval	Adult care home administrator certification

Whichever you choose, the fundamentals in this guide — screening, training, medications, resident rights, records, and funding — apply to both.

The Path at a Glance

1. Decide your model — family care home (2–6) or adult care home (7+) — and set up the business (LLC, EIN, register with the NC Secretary of State).
2. Check bed need. Adult care home beds are governed by the annual State Medical Facilities Plan and may require a Certificate of Need; confirm before you commit to a larger home.
3. Secure a location and confirm zoning with the local zoning board.

4. Contact your county DSS adult home specialist — the front door, especially for a family care home.
5. Complete building, fire, and sanitation review with the DHSR Construction Section and local building inspector, fire marshal, and sanitarian.
6. Submit the initial license application (DHSR/AC 4603) with the \$315 fee; your legal entity is verified with the Secretary of State.
7. Get the administrator certified (adult care home) or approved (family care home) and hire and screen staff.
8. Adopt your policies and procedures, set up resident records, and get survey-ready.
9. Pass the on-site inspection and receive your license.
10. Enroll as a Medicaid provider for Personal Care Services and connect eligible residents to Special Assistance — then open.

Part 1 — Choose Your Model and Set Up the Business

Start by choosing your size. A family care home (2–6 residents) is the most common way an individual enters this field: lower startup cost, a house-scale operation, and a process that runs through your county DSS. An adult care home (7+ residents) is a larger business with more staffing and physical-plant requirements and the added step of bed-need review. Your choice shapes everything that follows, so decide it first.

Form your business as you would any care company: create your legal entity (most owners form an LLC), get an EIN from the IRS, and register with the North Carolina Secretary of State — DHSR verifies your entity's status with that office during licensing. Set up business banking and confirm what business privilege or local licenses your city or county requires. If you plan to bill Medicaid, keep your entity information consistent everywhere, because it must match across your license, your Secretary of State record, and your Medicaid enrollment.

Protect the business early. An LLC helps separate your personal assets from the company, and you should line up the insurance a care operation needs — typically general liability and, depending on your situation, professional liability and property coverage — before you admit residents. Talk to a commercial insurance broker who understands residential care, and keep proof of coverage on file. Building these protections in from the start is far cheaper than adding them after a problem.

Part 2 — Bed Need, Zoning and Location

For adult care homes, North Carolina controls how many new beds are added. Each year the State publishes a State Medical Facilities Plan that identifies where adult care home beds are needed, and adding beds can require going through the Certificate of Need review on the schedule set in that plan. Before you sign a lease or buy a building for a seven-plus-bed home, confirm the bed-need and Certificate of Need situation for your county. Family care homes generally follow the county DSS process rather than this bed-need review, but confirm your specific situation.

Location is not just real estate — it is code. Confirm with your local zoning board that a licensed care home is permitted at the address, since the county DSS adult home specialist will refer you to zoning early. Then think ahead to the building requirements in Part 4: the home must meet the North Carolina State Building Code for its license type and capacity, and the number of residents you can serve depends on their ability to evacuate and on features such as sprinkler protection. A building that looks fine can still cap your capacity, so involve the DHSR Construction Section before you commit.

Part 3 — The County DSS Front Door and DHSR

For a family care home, your first call is to the adult services section of your county Department of Social Services and its adult home specialist, who answers licensing questions and guides applicants through the process. In an initial interview the specialist reviews the licensure rules and process, refers you to the local zoning board, and refers you to the DHSR Adult Care Licensure Section to request the application materials. The specialist then collects your completed materials and sends them to DHSR to finish licensing.

For an adult care home, you work directly with the DHSR Adult Care Licensure Section for the application while still coordinating with the county on assessment and Special Assistance. Either way, keep both relationships warm: DHSR holds your license and surveys you annually, and the county

monitors you at least quarterly and handles the assessments and Special Assistance that bring you residents and revenue.

Part 4 — Building, Fire and Sanitation Review

Plan review is where many timelines slip, so start it early. You submit plans to the DHSR Construction Section (for new construction, scaled drawings). After any construction or renovation, you send the Construction Section the approval documents from your local building inspector, fire marshal, and sanitarian, plus anything else the plan reviewer requests. The Construction Section then completes an on-site inspection and issues a building transmittal that states your allowable bed capacity, your residents' evacuation capability, and any building-related restrictions, and sends it to the Adult Care Licensure Section.

Three local sign-offs matter most: the building inspector (the structure meets code), the fire marshal (fire-safety systems, exits, and drills), and the sanitarian or environmental health (kitchen, water, and sanitation). Line these up in parallel where you can, and design to the code from the start — features like sprinkler protection and exit design directly determine how many residents, and how ambulatory, you may serve.

Part 5 — The Application and the Administrator

The license application is the Initial Application for licensure (form DHSR/AC 4603), submitted with a licensure fee of \$315. It identifies your legal entity and the individuals responsible — for an LLC, the managing members — and DHSR verifies the entity with the Secretary of State and runs a compliance-history check under G.S. 131D-2.4. Unless there are unresolved operational issues, incomplete information, or a disqualifying compliance history, DHSR proceeds toward issuing the license after a satisfactory on-site review.

Every home must be run by a qualified administrator. An adult care home administrator must hold North Carolina Adult Care Home Administrator certification; a family care home administrator must hold family care home administrator approval. Plan for this early, because certification takes time and the license depends on it. Keep the administrator's certification, continuing education, background check, and immunization records on file from day one.

Part 6 — Staffing, Training and Background Checks

Before anyone works with residents, North Carolina requires screening: a criminal background check under G.S. 131D-40 and a check of the Health Care Personnel Registry, and you may not employ a person with a disqualifying registry finding. Staff also need documented annual influenza immunization under G.S. 131D-9 and tuberculosis screening. At all times at least one staff person 18 or older must be in charge of resident care, and you must have enough qualified staff for your census and your residents' needs.

Training is not optional. Direct-care staff complete personal care training and a competency check before working independently. Medication aides must complete the required training, a clinical skills validation, and the state medication aide examination administered by DHSR, plus continuing education — only qualified staff may pass medications. Build a simple system now to track each person's

screening, immunizations, training, and competencies, because surveyors check it and lapses are among the most common citations.

Remember that training is ongoing, not a one-time event. Staff need at least annual training on infection control consistent with CDC guidelines, on residents' rights and abuse prevention and reporting, and on your emergency procedures, and medication aides carry continuing-education requirements. New duties, new equipment, or a resident with new needs should trigger fresh training and a competency check. A credential-and-training tracker — who is current on CPR, medication-aide status, background rechecks, immunizations, and annual training, and what is expiring — keeps you survey-ready and is worth setting up before you open.

Part 7 — Policies, Records and Getting Survey-Ready

You must have written policies and procedures covering resident care and facility operation, kept current and available to staff and DHSR. This is exactly what the PolicyReady Adult Care Home & Family Care Home Policy & Procedure Manual provides, mapped to 10A NCAC 13F and 13G. Adopt it, tailor it to your home, train your staff on it, and review it at least annually.

Set up resident records before your first admission. Each resident needs admission information and an admission agreement, the resident assessment and individual care plan, physician orders and a medication administration record, progress and incident notes, the signed Declaration of Residents' Rights, and immunization documentation. Post the residents' rights, the grievance procedure with Ombudsman contact information, and your emergency procedures. Being survey-ready on day one — organized records, posted rights, completed drills — is the difference between a clean first survey and an early plan of correction.

Part 8 — Getting Paid: Special Assistance and Medicaid

Here is the honest money picture. North Carolina's main Medicaid program does not pay for assisted living the way it pays for a nursing home, so a licensed adult care home is funded by a combination: a cash supplement toward room and board, a Medicaid benefit for hands-on care, and private pay for the rest. Two public programs do the heavy lifting.

State-County Special Assistance (SA) is a cash supplement, administered by county DSS, that helps low-income residents pay the room-and-board cost of an approved adult care home. Medicaid Personal Care Services (PCS) is a Medicaid State Plan benefit that pays for hands-on help with activities of daily living — bathing, dressing, toileting, mobility, eating — for eligible residents, but never for room and board. A resident often has both, plus private pay. Importantly, if you accept Special Assistance residents you must accept the state SA rate.

Funding at a glance (verify current figures with county DSS and NC Medicaid — rates change annually):

Program	What it pays for	Amount	Key conditions
State-County Special Assistance — standard	Room & board in an adult care home	Up to \$1,359 / month (2025), plus \$70 / month personal needs allowance	COLA-adjusted yearly; home must accept the state rate; SA recipients are auto-eligible for Medicaid
Special Assistance — special care unit	Room & board in a licensed Alzheimer's/dementia special care unit	Up to \$1,743 / month (2025), plus \$70 / month allowance	For licensed special care units; COLA-adjusted
Medicaid Personal Care Services (PCS)	Hands-on help with activities of daily living (not room & board)	Daily per-diem rate (in 2025 PCS moved from 15-minute units to a daily per diem); billed under code 99509 with modifiers	Independent assessment required; Clinical Coverage Policy 3L-1; no waitlist for eligible residents
Private pay / other	The balance of care and housing costs	Market rate (varies by region and care level)	Neither public program covers the full cost; long-term-care insurance and VA benefits may apply

To qualify a resident for Special Assistance who does not receive SSI, countable income generally must be under about \$1,428.51/month and countable assets under \$2,000 (2025 figures) — confirm current limits with county DSS. Apply for Special Assistance and Medicaid through the county DSS; PCS eligibility involves a separate independent assessment of the resident's care needs.

Becoming a Medicaid Personal Care Services Provider

Special Assistance flows through your county DSS, but to collect the Medicaid Personal Care Services (PCS) that pay for hands-on care, your home has to be enrolled as a Medicaid provider and each resident's care has to be authorized. Here is the shape of it — confirm the current steps with NC Medicaid before you rely on them, because enrollment details change.

First, enroll as a North Carolina Medicaid provider through NCTracks, the state's provider enrollment and claims system. You will need your legal entity information, your adult care home license, and the usual provider credentialing details, and your entity information must match your license and Secretary of State record exactly. Once enrolled, you can submit PCS claims and receive payment for eligible residents.

Second, each resident's PCS must be authorized through an independent assessment. PCS eligibility and the authorized amount are determined by an independent assessment conducted by NC Medicaid or its designee, based on the resident's need for hands-on help with activities of daily living, and the care is delivered according to an individual service plan. There is no waitlist — an eligible resident is entitled to the service — but you cannot bill for care that has not been assessed and authorized.

Third, bill correctly. PCS in adult care homes is billed under Clinical Coverage Policy 3L-1 using procedure code 99509 with the applicable modifiers, and in 2025 reimbursement for residents in congregate settings moved from fifteen-minute units to a calculated daily per-diem rate. Keep documentation that supports the care you provide, because Medicaid can review it. Plan your cash flow around the reality that enrollment, assessment, and the first payments take time — this is why working-capital reserves matter in your startup budget.

Special Care Units: Serving Residents with Dementia

A large share of North Carolina adult care homes operate a special care unit for residents with Alzheimer's disease and related dementias, and some homes are licensed as standalone special care unit facilities. If you intend to serve residents with significant dementia, decide early whether you will operate a special care unit, because it shapes your building, your staffing, your training, and your disclosures.

A home that holds itself out as providing special or Alzheimer's care is generally required to make additional written disclosures about what makes its care 'special' — the philosophy, staffing, training, activities, and physical environment — and to meet added expectations for a secure, supportive setting and dementia-specific staff training. Confirm the current special care unit requirements with DHSS before you market a unit as memory care, because describing your home as a special care unit carries real obligations.

There is a funding side to this decision. The Special Assistance special-care-unit rate is higher than the standard rate — up to \$1,743 per month in 2025 versus \$1,359 for a standard adult care home placement — reflecting the additional care these residents need. If dementia care is your model, building it correctly from the start lets you serve those residents well and be paid appropriately for it.

After You Open: Surveys, Ratings and Staying Compliant

Your license is the start, not the finish. DHSR surveys adult care homes on an annual basis and in response to complaints, and your county DSS monitors your home at least quarterly and investigates complaints. Treat continuous readiness as part of daily operation: keep licenses and policies current, resident records complete, personnel files (background checks, immunizations, training) up to date, and fire drills and equipment inspections on schedule. The homes that do well on survey are not the ones that scramble beforehand — they are the ones whose everyday records already tell the story.

North Carolina also gives adult care homes a star-rated certificate under G.S. 131D-10, based on how the home performs against standards including residents' rights, physical plant, staff qualifications, admission and discharge, assessment and care planning, and resident care and services. Your rating is public and shapes how families and referral sources see you, so it is worth managing deliberately. When a survey cites a deficiency, you submit a plan of correction within the required timeframe, fix the root cause, and sustain the fix — a thoughtful, well-implemented plan of correction protects both your residents and your standing.

Build a simple compliance rhythm: a calendar of recurring obligations (license renewal, background and registry rechecks, immunizations, training, drills, inspections, and annual policy review), a monthly review of incidents, medication errors, and grievances, and an annual self-assessment against your policy manual and the rules. This is exactly what a good policy and procedure manual sets up for you.

Finding and Admitting Residents

A licensed, empty home does not pay the bills, and in adult care the residents come through a defined set of channels rather than walk-in traffic. Your county DSS is central: it completes the pre-admission assessment, administers Special Assistance, and is often aware of individuals who need placement. Build that relationship early and keep your availability and the kinds of residents you serve current with the adult home specialist.

Beyond county DSS, referrals come from hospital discharge planners and social workers, skilled nursing and rehab facilities discharging residents to a lower level of care, physicians and clinics, the Long-Term Care Ombudsman and Area Agency on Aging, and families searching directly. The homes that stay full are easy to reach, respond the same day, are honest about who they can serve well, and keep discharge planners informed. Make sure your home is listed and accurate wherever families and professionals look, and that your first response to any inquiry is fast and clear.

Admit only residents whose assessed needs you can meet within your license, complete the required assessment and a written admission agreement, provide the Declaration of Residents' Rights, and start the resident's records and care plan from day one. A smooth, well-documented admission is both a compliance duty and your best marketing to the referrer who sent them. (PolicyReady also offers a referral system built specifically for filling your beds through these channels.)

What It Costs to Start

Exact startup cost depends heavily on your model and building, so treat the categories below as a planning checklist rather than a fixed number, and get real quotes for your situation. The single most variable cost is bringing a building to code for your license type and capacity.

- **Business setup.** LLC formation, EIN, NC Secretary of State registration, business banking, and any city/county business license.
- **Building to code.** Purchase or lease, plus renovation to meet the building code, fire-safety, and sanitation requirements for your capacity — usually the largest and most variable cost.
- **Licensing.** The initial application fee of \$315, plan-review costs, and local permit fees.
- **Administrator.** Administrator certification (adult care home) or approval (family care home) and continuing education.
- **Staffing and training.** Recruiting, background checks and registry checks, immunizations, personal care training, and medication-aide training and the state exam.
- **Operations and furnishings.** Beds and furnishings, kitchen and food service, supplies, a call system where required, and initial food and household stock.
- **Insurance and reserves.** General liability and other required coverage, and working-capital reserves for the months before Special Assistance and Medicaid payments stabilize your cash flow.

Common Mistakes to Avoid

- **Committing to a building before plan review.** Evacuation capability, sprinklers, and code can cap your capacity or require costly changes — involve the DHSR Construction Section before you buy or sign.
- **Skipping the county DSS front door.** For a family care home, the adult home specialist is your guide and collects your application — starting anywhere else slows you down.
- **Underestimating administrator certification time.** The license depends on a certified or approved administrator; start that process early.
- **Hiring before screening.** Background checks and the Health Care Personnel Registry check must clear first, and a disqualifying finding bars employment.
- **Treating medication aide rules loosely.** Only staff who completed training, the clinical skills validation, and the state exam may pass medications — medication errors and untrained staff are frequent, serious citations.
- **Assuming Medicaid covers room and board.** It does not — plan your finances around SA for room and board, PCS for care, and private pay for the gap.
- **Opening without survey-ready records and postings.** Missing care plans, MARs, posted rights, or fire drills lead to an early plan of correction.

Your First 90 Days

Days 1–30: finalize your entity and administrator, lock in your location and zoning, and open the plan-review conversation with the DHSR Construction Section and your county DSS adult home specialist. Begin any construction or renovation and line up the building, fire, and sanitation sign-offs.

Days 31–60: submit your application (DHSR/AC 4603) with the \$315 fee, recruit and screen staff, and complete required training and medication-aide qualification. Adopt and tailor your policy and procedure manual, and stand up your resident-records and medication systems.

Days 61–90: complete construction sign-offs and the on-site inspection, correct anything identified, and receive your license. Enroll as a Medicaid PCS provider, build your county DSS relationship for assessments and Special Assistance, do a mock survey against your manual, and admit your first residents with complete records from day one.

Local Call List — Charlotte (Mecklenburg County)

Verified starting contacts for opening in the Charlotte area. Confirm the right division and current number when you call.

Agency	Why you call them	Phone
Mecklenburg County DSS (Adult Services)	Family care home front door; adult home specialist; Special Assistance & assessments	704-336-3000
Mecklenburg County Code Enforcement	Building permits and inspections	704-336-7600
Mecklenburg County Environmental Health	Kitchen, water and sanitation (sanitarian) sign-off	704-336-7600
City/County Zoning (via Code Enforcement)	Confirm a licensed care home is permitted at your address	704-336-7600

For fire-safety sign-off, contact the fire marshal for your jurisdiction (city of Charlotte or your town). Your regional Long-Term Care Ombudsman for Mecklenburg is housed in the Centralina Area Agency on Aging — reach it through the State Ombudsman line in the statewide table.

Local Call List — Raleigh (Wake County)

Verified starting contacts for opening in the Raleigh area. Confirm the right division and current number when you call.

Agency	Why you call them	Phone
Wake County Health & Human Services (DSS/Adult)	Family care home front door; adult home specialist; Special Assistance & assessments	919-212-7000
Wake County Building Inspections / Planning	Permits, plan questions, county inspections	919-856-6310
City of Raleigh Inspections Department	City building permits and inspections	919-996-2500
Wake County Environmental Services	Sanitation / environmental health (sanitarian)	919-856-7400

For fire-safety sign-off, contact the fire marshal for your jurisdiction (city of Raleigh or your town). Your regional Long-Term Care Ombudsman for Wake is housed in the Triangle J Area Agency on Aging — reach it through the State Ombudsman line in the statewide table.

Local Call List — Greensboro (Guilford County)

Verified starting contacts for opening in the Greensboro/High Point area. Confirm the right division and current number when you call.

Agency	Why you call them	Phone
Guilford County DSS (Adult Services)	Family care home front door; adult home specialist; Special Assistance & assessments	336-641-3000

Guilford County Building Inspections	Permits and building inspections	336-641-3707
Guilford County Environmental Health	Kitchen, water and sanitation (sanitarian) sign-off	336-641-7613

For fire-safety sign-off, contact the fire marshal for your jurisdiction (city of Greensboro, High Point, or your town). Your regional Long-Term Care Ombudsman for Guilford is housed in the Piedmont Triad Regional Council Area Agency on Aging — reach it through the State Ombudsman line in the statewide table.

Statewide Contacts

Office	Purpose	Phone
DHSR Adult Care Licensure Section	Licensing, rules, surveys (1915 Health Services Way, Raleigh)	919-855-3765
NC Division of Aging & Adult Services	Aging services; houses the State Ombudsman	919-855-3400
State Long-Term Care Ombudsman	Residents' rights; regional ombudsmen via Area Agencies on Aging	919-855-3426
NC Medicaid (Division of Health Benefits)	Medicaid eligibility and program information	888-245-0179
NC Medicaid Ombudsman	Help for Medicaid beneficiaries	877-201-3750
NC Medicaid Provider Ombudsman	Provider inquiries and concerns	866-304-7062
NC DOJ Medicaid Investigations Division	Report Medicaid provider fraud or patient abuse	919-881-2320
Adult Protective Services	Report abuse, neglect, exploitation (via county DSS)	Your county DSS

Quick Glossary

- **Adult care home (ACH).** A licensed residence serving 7+ adults, under 10A NCAC 13F.
- **Family care home (FCH).** A licensed residence serving 2–6 adults, under 10A NCAC 13G — the small-home path.
- **DHSR / ACLS.** Division of Health Service Regulation and its Adult Care Licensure Section — they license and survey your home.
- **County DSS / adult home specialist.** Your local partner; the family care home front door; administers Special Assistance and assessments.
- **Special Assistance (SA).** A cash supplement toward room and board for eligible low-income residents.
- **Personal Care Services (PCS).** The Medicaid benefit that pays for hands-on ADL care, not room and board.
- **Certificate of Need (CON).** State review that can control the addition of adult care home beds, per the State Medical Facilities Plan.
- **Medication aide.** Staff qualified by training, clinical skills validation, and the state exam to pass medications.

Core Rules to Keep Close

- G.S. 131D and 10A NCAC 13F (adult care homes) / 13G (family care homes) — licensing and operation.
- G.S. 131D-21 — Declaration of Residents' Rights.
- G.S. 131D-4.5 and S.L. 2011-99 — medication administration and infection control.

- G.S. 131D-40 and G.S. 131E-256 — criminal background checks and the Health Care Personnel Registry.
- G.S. 131D-9 — staff influenza immunization.

This guide is general educational information for prospective North Carolina adult care home and family care home operators and is not legal advice. Rules, fees, figures, and contacts change — verify each with DHSR, your county DSS, and NC Medicaid before relying on it. Pair this guide with a current policy and procedure manual and a complete forms set, and confirm your specific obligations before you open.